

Bipolar Disorder: Patient Information Leaflet

Defined by at least one full manic episode — a sustained period of elevated or irritable mood, high energy, and impaired judgment — usually alongside episodes of depression.

What is Bipolar I Disorder?

Bipolar I Disorder is defined by the occurrence of at least one manic episode: a distinct period of abnormally elevated, expansive, or irritable mood combined with a marked increase in energy and activity, severe enough to cause significant impairment, require hospitalization, or involve psychotic features. While most people with Bipolar I also experience major depressive episodes over their lifetime, depression is not required for the diagnosis — a single manic episode is sufficient. Between episodes, many people return to their usual level of functioning, though the illness carries a substantial risk of relapse and, notably, one of the highest suicide risks of any psychiatric condition.

Common symptoms

- Manic episode: elevated, expansive, or irritable mood with high energy
- Inflated self-esteem or grandiosity
- Decreased need for sleep (e.g., feeling rested after only 3 hours)
- More talkative than usual, or pressured speech
- Racing thoughts or flight of ideas
- Distractibility
- Increased goal-directed activity or psychomotor agitation
- Excessive involvement in risky activities (spending sprees, impulsive sexual behavior, risky investments)

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Bipolar I Disorder has one of the strongest genetic contributions among major psychiatric conditions, with heritability estimated at 60–80%. It is thought to arise from dysregulation of dopamine signaling (particularly during manic episodes) and disruption of the brain's circadian rhythm regulation. According to the 'kindling' model, episodes can become more frequent and increasingly spontaneous (requiring less external trigger) over time if the illness goes untreated, which is part of why early, sustained treatment matters.

Treatment options

Mood stabilizers form the cornerstone of both acute and long-term treatment, with lithium remaining the gold-standard option, notably also reducing suicide risk. Acute severe mania, especially with psychotic features, often requires an atypical antipsychotic and may require hospitalization for safety. Antidepressants are generally avoided as monotherapy, since they carry a risk of triggering mania or accelerating episode cycling if not combined with a mood stabilizer.

Self-help tips

- Maintain a strict, regular sleep schedule — sleep deprivation is one of the most powerful triggers of mania
- Avoid alcohol and recreational drugs, which can destabilize mood
- Track mood, sleep, and energy daily to notice early warning signs
- Maintain consistent daily routines and manage stress proactively
- Prioritize medication adherence even when feeling well, since stopping treatment is a leading cause of relapse

Prognosis

Bipolar I Disorder is a chronic, relapsing condition, but with consistent treatment — especially lithium or another mood stabilizer — many people achieve good long-term functional stability. Left untreated, the illness tends to have a high relapse rate and can lead to progressive functional decline. Suicide risk is significantly elevated

compared to the general population, making ongoing treatment and monitoring

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When to seek urgent help

Seek immediate emergency care for severe mania with psychotic features, any suicidal thoughts during a depressive phase, dangerous or risky behavior threatening safety (reckless spending, sexual risk-taking, aggression), or an inability to care for basic needs during a severe episode.

This material is for patient and family education only and does not replace individualized medical advice. For a personal diagnosis or treatment plan, consult Dr. Kushal Kharel or a qualified mental health professional.

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